One Drug, Many Prices:
A Proposal to Reform US Healthcare Pricing

Introduction

The US healthcare pricing system is broken: the same product is sold to different payers for wildly different amounts, patients cannot compare or even know the price of their care beforehand, pharmacy benefit managers coerce payment of undisclosed sums for themselves and hide the amounts as a tightly guarded secret, and the largest public insurer is largely prohibited from using its leverage to reduce prices.

I present three interconnected reforms that combine to offer a comprehensive solution based on existing tools: consolidated government negotiation, Bloc-wide pricing as a condition of market access, and a central price lookup system. I describe multiple cases in which some variant of these reforms has been implemented with promising results. This note is an intellectual exercise in policy design, addressed to academics and policy makers. Its purpose is to present an idea for consideration, clarify its logic, and invite critique and refinement. It leaves to others the work of political strategy, legislative drafting, and advocacy.

A colonoscopy costs $880 at the VA, $1,760 under Medicare, $5,816 under a commercial insurer, and $19,206 for uninsured patients. A chest X-ray costs $20 under Medicare, $60 under a private insurer, and $283 to the uninsured. These are not anomalies. Figure 1 shows the same pattern across ten commonly used drugs, hospital services, and outpatient procedures. In every case, the price of the same product or service swings several-fold across payers.

Uninsured patients are a vulnerable group who often face the highest prices. They pay the chargemaster prices in hospitals, fill prescriptions at undiscounted rates, and are billed whatever a physician's office chooses to post.

Until the Inflation Reduction Act of 2022, Medicare was generally not allowed to negotiate

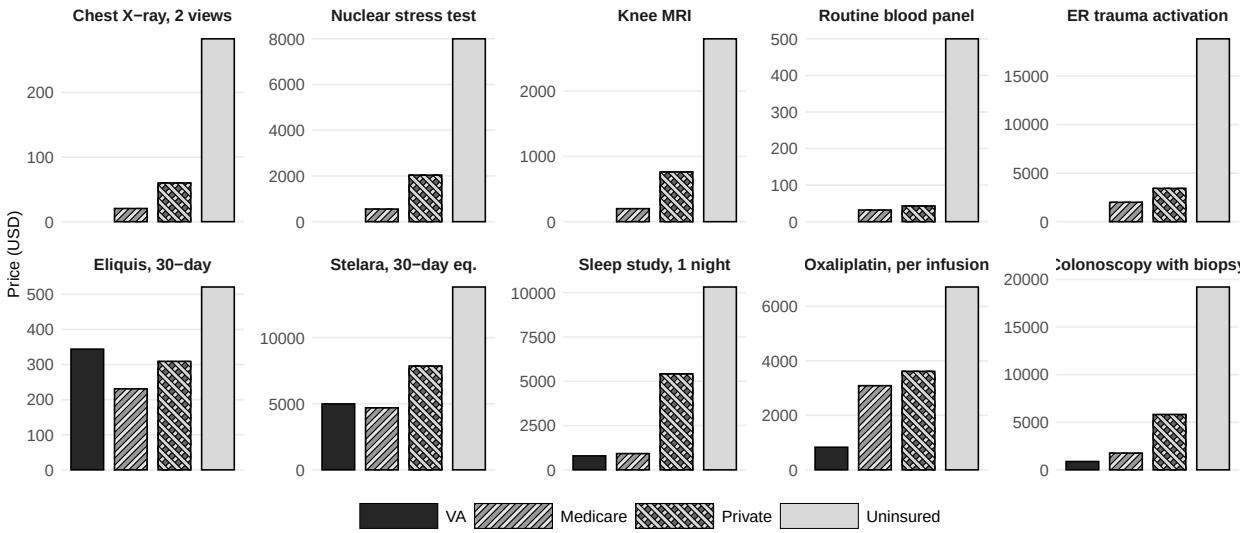

Figure 1: Price gaps across payers for the same service or product.

Notes: Bars show the total price each payer faces for the same service or product. The "private insurer" bar is the total negotiated rate, including patient cost-sharing; the "uninsured" bar is the chargemaster or list price. Each panel uses its own vertical scale. Missing bars indicate that a reliable figure was not available in the cited sources. Full sources and derivations for each number are on the companion site.

drug prices. The Act authorized negotiation for ten drugs with the highest Medicare spending. Figure 2 compares prices in the US and six other developed countries for nine of those ten drugs. Insulin aspart is excluded due to reporting incompatibility across countries. The US list price towers over what every peer nation pays. Even after rebates and discounts, private insurers in the US pay multiples of what patients in Canada, France, or the UK pay for the same drug. Medicare's newly negotiated prices narrow the gap but remain well above the rest of the developed world.

Americans usually do not know the price at the time of receiving care. Prices are either unavailable or difficult to obtain. This lack of transparency is a key driver of the price dispersion shown above. The secrecy is not incidental. It is structural. No other developed nation operates a pricing system this opaque.

Two economic forces determine an insurer's ability to bring down healthcare prices. The first factor is insurer size. Larger insurers have more leverage because losing the insurer's high volume is more painful for the provider. The second factor is the availability of alternative insurers, which allows the provider to exclude insurers that demand low prices. Walking away is not an option when the insurer is irreplaceable. The two forces work in opposition. A large insurer surrounded by many competing insurers has weaker leverage than a smaller insurer that is the only game in town. The leverage is maximum when the insurer is large and difficult to replace. The three reforms below are designed around this logic. Consolidated

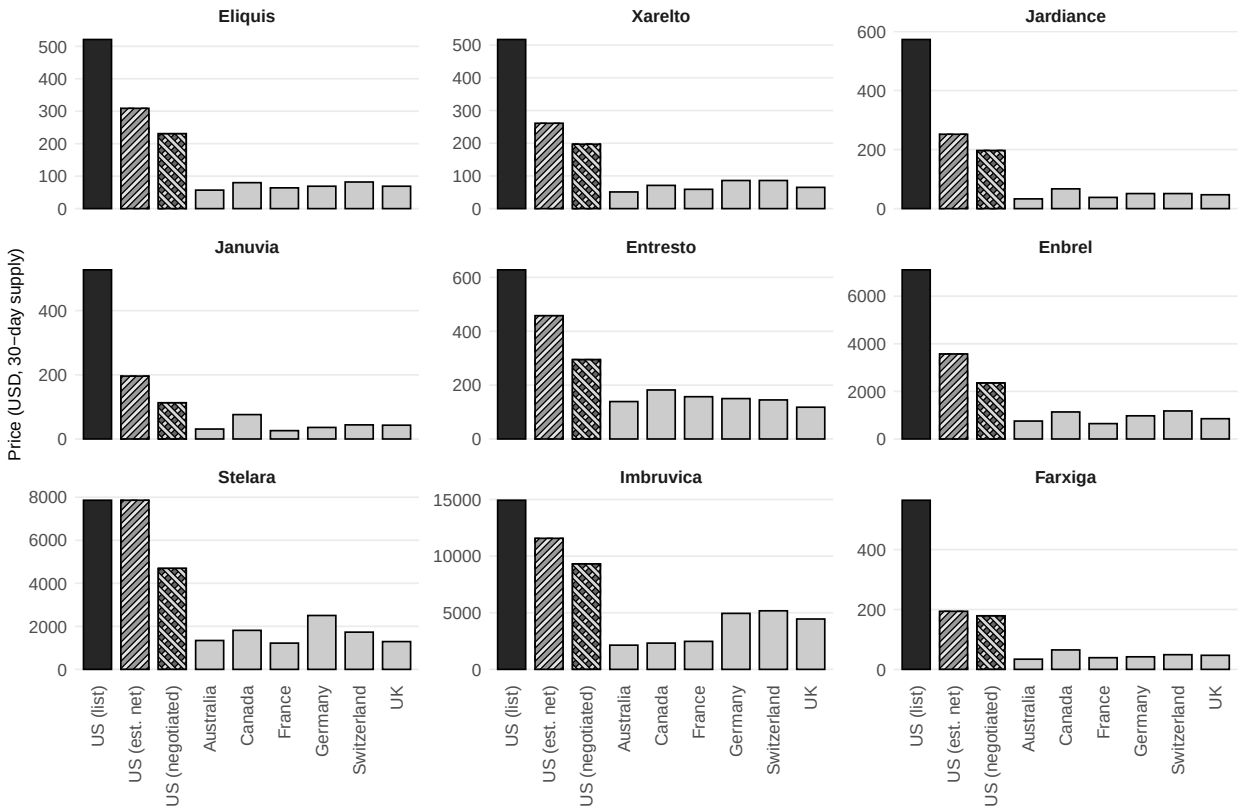

Figure 2: Drug prices by country.

Notes: The solid dark bar is the US list price (paid by the uninsured). The striped bar is the estimated US net price across all US payers combined, after rebates and discounts, derived from SSR Health data and SEC filings.[1] The crosshatched bar is the Medicare negotiated price under the Inflation Reduction Act (effective 2026). Each panel uses its own vertical scale. Full per-drug figures and sources are on the companion site.

negotiation makes the government a single, large insurer. Bloc-wide pricing eliminates the alternative insurers that providers could otherwise turn to for higher rates. A central price lookup system ensures the prices that emerge from negotiation are honored, not undercut by the secret arrangements that historically diluted government leverage.

Reform 1: Consolidated government negotiation

The federal government does not negotiate healthcare prices as a single insurer. It fragments its purchasing power across Medicare, Medicaid, and at least nine other programs, each with its own formulary, fee schedule, rules, and leverage. For prescription drugs alone, these programs account for 51% of US spending, yet they compete against one another instead of bargaining as one.[2] The government pays a similarly large share of hospital and physician bills, and those payments are just as fragmented. Consolidating these programs into a single negotiating Bloc, a unified entity that bargains on behalf of all federal healthcare programs, would use leverage the government has but wastes. Other developed nations leverage their

full size in what resembles a bloc, and pay far less.[3]

When the government has the power to lower healthcare costs, it has a duty to use it. For most of its history, Medicare itself was statutorily prohibited from negotiating drug prices. This arbitrary restriction is indefensible, lacks policy rationale, and is unparalleled in the developed world. The Inflation Reduction Act of 2022 began to reverse it, authorizing Medicare to negotiate prices for ten high-expenditure drugs. Even confined to ten drugs, the negotiation lowered Medicare's net prices by roughly 22% on average, with reductions reaching beyond 30% on the highest-cost drugs.[4] The government has the means to negotiate all prices at once. Confining that effort to ten drugs squanders the leverage and wastes taxpayer money. In contrast, the VA is not statutorily prohibited from negotiating. Despite its much smaller size, the freedom to negotiate lets it pay roughly half of what Medicare Part D pays.[5]

Reform 1 outlaws rebates, chargebacks, spread pricing, and side payments of any kind. Negotiation is conducted by salaried Bloc negotiators with expertise in pharmacology, clinical medicine, and health economics. The same negotiators make formulary and tiering decisions, since a drug's tier is a negotiating instrument and a drug's price determines whether it deserves preferred placement. Bloc negotiators are prohibited from accepting any payment beyond their government compensation package. This prohibition covers consulting fees, speaking fees, seminar honoraria, gifts, hospitality, and private-sector employment in any capacity. For twelve months after leaving the Bloc, they are also barred from employment in any sector of the regulated healthcare industry. Compensation follows the federal Senior Executive Service model: a fixed salary at the SES level and a performance bonus capped at 10% of base, awarded on periodic peer-reviewed evaluations rather than per-contract outcomes. The companion site discusses the empirical literature supporting this incentive structure over either pure salary or performance-based pay.

In short, Reform 1 is to pool all federal healthcare purchasing into one Bloc that negotiates prices for every covered drug, hospital service, and physician service on behalf of every federal program.

Reform 2: Bloc-wide pricing as a condition of market access

Any provider that contracts with the Bloc retains the freedom to choose which private insurers to contract with, but once a contract is signed, the price must be the same as the Bloc-negotiated rate. Uninsured patients cannot be refused or charged a different price. The negotiated price is locked in: no insurer can negotiate further discounts on the side. Any

provider that violates these terms loses its contract with the Bloc, and with it, access to the majority of the market.

Reform 2 does not make insurance plans identical; it standardizes the price, not the coverage. Once the price of a service is fixed, insurers would compete on what fraction of that price they cover and what fraction the patient pays out of pocket. Generous plans would cover most of the uniform price and leave the patient with a small copay. Cheaper plans would expose patients to a higher copay. Both would pay the provider the same; what would differ is how the cost is split between the insurer and the patient.

Reform 3: A central price lookup system

Reforms 1 and 2 produce one negotiated price per provider per service. Reform 3 makes that price visible. The Bloc operates a single, public, online price lookup system that serves as the canonical record of every contracted price in the country. Each entry pairs a service code with a provider and a price. The system covers the vast majority of all healthcare products and services, for all providers and pharmaceutical companies that contract with the Bloc. It updates whenever a contract is signed or revised.

Whenever transparency is left to the provider to deliver, whether through posting, reporting, or uploading, compliance becomes the weak link. Misreporting and gaming become systemic, and enforcement chases thousands of moving targets. There is no need to wait for the provider. The mere existence of a contract between the Bloc and the provider implies that the price of every covered service is pre-set, whether per service or per bundled episode. All that remains is to transfer those prices from the contract to the database, and Bloc officials do that directly, if not automatically. Removing the reliance on the provider's input would close many loopholes for gaming the system.

The lookup system is open to anyone, free, and built for comparison. No registration or identity verification is required. A patient who needs an ENT visit can see the price at every provider in their area. A patient considering a hip replacement can compare every hospital within a chosen radius. A parent who needs an EpiPen for their child can see the price at every nearby pharmacy in real time. The Bloc may also publish smartphone applications to help patients search and compare providers and prices. The system provides a public API for systematic queries, allowing third parties to build comparison tools, integrate prices into electronic health records and patient portals, and develop applications that may serve patients better than anything the government produces directly.

Physicians can use the same tool when prescribing. A doctor writing a prescription for an

expensive drug can identify the pharmacy with the lowest local price and send the prescription there. A patient facing a costly procedure can be referred to a lower-priced provider, even one across state lines if the savings warrant the trip. This would intensify competition: physicians, more informed and more influential than patients in directing demand, would become active routers of patients to lower-priced sellers.

Enforcement follows directly. Every patient charged more than the listed price would have the receipts to file a complaint, and the Bloc would have the matching contract on file. The fine would be high enough to deter overcharging. Because few patients are likely to file complaints over modest overcharges, the multiplier must be large; a figure on the order of 100 times the overcharge may be appropriate. The system should be designed to terminate contracts with repeat offenders, regardless of whether fines are paid. Existing transparency rules, such as the Hospital Price Transparency Rule and the Transparency in Coverage Rule, have largely failed to deliver usable transparency precisely because they delegated posting to thousands of providers and imposed symbolic penalties. Hospitals that ignore those rules face fines as low as $300 per day, a trivial sum for a system billing millions.[6] A central lookup system makes reporting automatic and accurate, consolidates all data into one comparison system, and imposes meaningful penalties and termination clauses to deter noncompliance.

What the reforms mean for each stakeholder

Patients would be the biggest beneficiaries of these reforms. They would face lower prices, comparison-shop among providers, and know the price before receiving care. Insured patients would pay lower premiums because the prices underlying them would fall, and uninsured patients would pay the same listed price as everyone else.

Providers and hospitals could no longer maintain wide price gaps across payers. An average private insurer in 2022 paid 2.5 times the Medicare rate for hospital services, with the largest gaps in concentrated markets where a single hospital chain dominates.[7,8] Under the Bloc, that gap would collapse and price discrimination would disappear. Hospitals in concentrated markets would face the largest reductions, because they currently extract the largest markups.

The insurance plan market would survive, but competition would shift from negotiating secret prices to designing coverage. Narrow networks would lose their purpose: today they exist because insurers extract lower rates from providers in exchange for steering patient volume. When every provider charges the same price, restricting network size would offer no advantage.

The rebate ecosystem would cease to exist, and the drug supply chain would undergo an

overhaul that would eliminate many unnecessary intermediaries. There would be no rebates to capture, no spread pricing to exploit, and no opaque price differentials to arbitrage. Pharmacy benefit managers, whose business model depends on these arrangements, would lose their function.

Related literature and existing implementations

None of the three reforms is new in isolation. Each has been studied, advocated, or partially implemented. The contribution of this note is to argue that they are mutually reinforcing, that each closes a loophole the others leave open, and that the three together produce results neither component can produce alone. The literature and precedents behind each reform are summarized below.

Reform 1 (consolidated negotiation). That the federal government leaves bargaining power on the table by fragmenting its purchasing across Medicare, Medicaid, the VA, the DoD, the IHS, and other programs has been documented repeatedly. The CBO reports that prices paid by federal programs for identical drugs differ by wide margins, with the VA paying roughly half of Medicare Part D for selected drugs.[9,10] Gusmano and colleagues show that peer nations achieve lower fee-for-service prices precisely by negotiating as unified blocs.[3] Consolidated purchasing already works at smaller scales inside the US federal system. The Federal Supply Schedule publishes VA-negotiated contract prices that the Department of Defense (including Tricare), the Indian Health Service, the Federal Bureau of Prisons, and the Public Health Service all purchase off,[11] demonstrating that a single negotiating body can serve many federal agencies at one negotiated price. At the state level, pooled purchasing coalitions such as the Sovereign States Drug Consortium extract supplemental Medicaid rebates of 5 to 15% beyond federal levels by combining state purchasing power.[12,13] Under the Inflation Reduction Act, Medicare's first exercise of negotiation, confined to ten drugs, cut net prices by roughly 22% on average.[4] Reform 1 scales the same mechanism from a dozen fragmented programs to one Bloc covering every federal healthcare purchase.

Reform 2 (uniform pricing as a condition of market access). The case for all-payer pricing has been argued in the US health-policy literature for decades. Reinhardt's sustained critique of multi-price systems showed that price discrimination across payers generates opacity, cost-shifting claims that do not hold up empirically, and persistent exploitation of the weakest payers.[14] Subsequent work confirmed the empirics: White and Frakt find little evidence that lower Medicare payments push private payments up, undercutting the industry's standard defense of the multi-price system.[15,16] Anderson and Herring set out the

design case for a US all-payer rate-setting framework drawing on the German, Swiss, and Japanese examples, and on Maryland's all-payer hospital system, which has operated since 1977.[17] Emanuel and coauthors advanced a version of the same proposal in a widely-cited *New England Journal of Medicine* piece.[18] Cooper and colleagues document the magnitude of what all-payer pricing would undo, namely private-insurer prices that run at roughly 2.5 times Medicare rates, with the largest gaps in concentrated hospital markets.[7,8] Conditioning federal market access on pricing behavior is also not new: the 340B Drug Pricing Program requires manufacturers that participate in Medicaid to sell outpatient drugs to safety-net providers at roughly 25 to 50% below list, and the Medicaid Best Price rule requires manufacturers to offer Medicaid the lowest price offered to any non-federal purchaser. Both show that conditional-access mechanisms are workable and durable. Reform 2 combines the all-payer tradition with the conditional-access enforcement tool: any provider that accepts the Bloc's patient pool must offer every other payer, and the uninsured, the same price.

Reform 3 (price transparency). Empirical evaluations of price-transparency interventions consistently find price reductions where transparency is actually achieved. The CalPERS reference-pricing program, which publishes prices and allows patients to compare providers, reduced prices by 17 to 26% and shifted patient volume toward lower-priced facilities.[19,20] New Hampshire's HealthCost tool produced modest price reductions for shoppable services, particularly imaging.[21] Singapore publishes hospital-bill benchmarks for common procedures by provider and has induced visible price competition. Federal transparency rules already exist in the US: the Hospital Price Transparency Rule (2021) and the Transparency in Coverage Rule (2020) require hospitals and insurers to post prices in machine-readable form. Both have largely failed to deliver usable transparency because they delegate posting to thousands of providers and insurers, impose symbolic penalties, and produce data too inconsistent to compare across sellers.[6] Reform 3 departs from these rules on a design point rather than a policy one: because every price is set in a Bloc contract, the Bloc, not the provider, is the natural publisher of record. This removes the weakest link in the existing transparency regime.

These three reforms are not proposed together anywhere in the literature I have found. Reform 1 without Reform 2 produces a powerful buyer facing private-pay loopholes that erode its gains, which is the current situation. Reform 2 without Reform 1 imposes a uniform rate with no one empowered to set it low. Either without Reform 3 remains exposed to the secret side-deals and misreporting that have defeated past price-control efforts. The contribution of this note is the claim that the three belong together, and the design that makes them enforceable as a single system.

Online appendix. A set of frequently asked questions covering bundled pricing, negotiator compensation, specialist opt-outs, rural hospitals, drug manufacturer withdrawal, fiscal impact, and related objections is maintained on the companion site. Comments, criticisms, and suggestions are welcome through the contact form there.

Disclosure on the use of AI. I used AI tools throughout the preparation of this manuscript to refine arguments, collect and verify data, produce figures and tables, build the companion website, and polish the prose. The ideas, intellectual contributions, and editorial judgments are mine, and I take full responsibility for the content.

References

[1] Olivier J. Wouters, Sean D. Sullivan, Emma M. Cousin, Nico Gabriel, Irene Papanicolas, and Inmaculada Hernandez. Drug prices negotiated by Medicare vs US net prices and prices in other countries. *JAMA*, 333(1):85–87, 2025.

[2] Centers for Medicare & Medicaid Services. National health expenditure accounts: Historical. Technical report, Centers for Medicare & Medicaid Services, 2024. URL https://www.cms.gov/data-research/statistics-trends-and-reports/national-health-expenditure-data/historical.

[3] Michael K. Gusmano, Miriam J. Laugesen, Victor G. Rodwin, and Lawrence D. Brown. Getting the price right: How some countries control spending in a fee-for-service system. *Health Affairs*, 39(11):1867–1874, 2020.

[4] Kaiser Family Foundation. Explaining the prescription drug provisions in the inflation reduction act: Negotiated prices for the first ten drugs. KFF Issue Brief, 2024. URL https://www.kff.org/medicare/issue-brief/explaining-the-prescription-drug-provisions-in-the-inflation-reduction-act/. Estimates net price reductions versus pre-negotiation net prices.

[5] Congressional Budget Office. Policy approaches to reduce what commercial insurers pay for hospitals' and physicians' services. Technical report, Congressional Budget Office, 2022. URL https://www.cbo.gov/publication/58222.

[6] Centers for Medicare & Medicaid Services. Hospital price transparency: Civil monetary penalty for noncompliance (45 cfr 180.90). CMS Regulation, 2021. URL https://www.cms.gov/hospital-price-transparency/enforcement-actions. Initial penalty set at $300 per day; raised in 2022 to $300–$5,500 per day depending on hospital size.

[7] Christopher M. Whaley, Rose Kerber, Daniel Wang, Aaron Kofner, and Brian Briscombe. Prices paid to hospitals by private health plans: Findings from round 5 of an employer-led transparency initiative. Technical report, RAND Corporation, 2024. URL https://www.rand.org/pubs/research_reports/RRA1144-2.html.

[8] Zack Cooper, Stuart V. Craig, Martin Gaynor, and John Van Reenen. The price ain't right? hospital prices and health spending on the privately insured. *Quarterly Journal of Economics*, 134(1):51–107, 2019.

[9] Congressional Budget Office. A comparison of brand-name drug prices among selected federal programs. Technical Report 56978, Congressional Budget Office, February 2021. URL https://www.cbo.gov/publication/56978.

[10] U.S. Government Accountability Office. Prescription drugs: Department of Veterans Affairs paid about half as much as Medicare Part D for selected drugs in 2017. Technical Report GAO-21-111, U.S. Government Accountability Office, January 2021. URL https://www.gao.gov/products/gao-21-111.

[11] Department of Veterans Affairs. VA National Acquisition Center Federal Supply Schedule pharmaceutical prices. U.S. Department of Veterans Affairs, 2026. URL https://www.va.gov/opal/nac/fss/pharmprices.asp.

[12] U.S. Department of Health and Human Services, Office of Inspector General. States' collection of offset and supplemental Medicaid rebates. Technical Report OEI-03-12-00520, HHS Office of Inspector General, December 2014. URL https://oig.hhs.gov/oei/reports/oei-03-12-00520.asp.

[13] Jane Horvath. State initiatives using purchasing power to achieve drug cost containment. Technical report, National Academy for State Health Policy, April 2019. URL https://nashp.org/wp-content/uploads/2019/04/Rx-Purchasing-Paper-Jane-Horvath-FINAL-4_9_2019.pdf.

[14] Uwe E. Reinhardt. The many different prices paid to providers and the flawed theory of cost shifting: Is it time for a more rational all-payer system? *Health Affairs*, 30(11):2125–2133, 2011.

[15] Chapin White. Contrary to cost-shift theory, lower Medicare hospital payment rates for inpatient care lead to lower private payment rates. *Health Affairs*, 32(5):935–943, 2013.

[16] Austin B. Frakt. How much do hospitals cost shift? a review of the evidence. *Milbank Quarterly*, 89(1):90–130, 2011.

[17] Gerard F. Anderson and Bradley Herring. The all-payer rate setting model for pricing medical services and drugs. *AMA Journal of Ethics*, 17(8):770–775, 2015.

[18] Ezekiel J. Emanuel, Neera Tanden, Stuart Altman, Scott Armstrong, Donald Berwick, François de Brantes, Maura Calsyn, David B. Cutler, Tom Daschle, and Gail Wilensky. A systemic approach to containing health care spending. *New England Journal of Medicine*, 367(10):949–954, 2012.

[19] James C. Robinson and Timothy Brown. Increases in consumer cost sharing redirect patient volumes and reduce hospital prices for orthopedic surgery. *Health Affairs*, 32(8):1392–1397, 2013.

[20] James C. Robinson, Timothy T. Brown, and Christopher Whaley. Reference-based benefit design changes consumers' choices and employers' payments for ambulatory surgery. *Health Affairs*, 34(3):415–422, 2015.

[21] Christopher M. Whaley. Provider responses to online price transparency. *Journal of Health Economics*, 66:241–259, 2019.